Office of Research Assurance

Human Subject Research Protections Standard Operating Procedures (SOP)

SOP - 430: Expedited Review

Number	Effective Date	Version Number	Previous Version
430	20-Aug-2025	v.3.3	v.3.2

Purpose

SOP-430 defines the NASA IRB criteria and processes related to reviewing new applications, modifications, and continuing review reports by expedited procedures. Expedited review allows the NASA IRB to review and approve studies that meet the criteria within this SOP without convening a meeting of the full NASA IRB.

NASA is an official signatory of the Common Rule codified at 14 CFR part 1230 and applies the Food and Drug Administration (FDA) regulations 21 CFR parts 50, 56, 312, 812 (as applicable) when projects involve activities regulated by the FDA.

Policy

1. **Review Authority**. Expedited review procedures may be conducted by the NASA IRB Chair or an experienced designated reviewer as stated in 14 CFR 1230.110 (b)(2) and FDA 21 CFR 56.110(b).
    a) **Designated Reviewer**: an experienced member of the NASA IRB authorized by the NASA IRB Chair to conduct expedited review.
    b) NASA IRB Chair/Designated Reviewers must not participate in the review of any research in which they have a conflict of interest.
        i) IRB Chair/Designated Reviewers who have conflict of interest (COI) related to a submission must inform the NASA IRB office staff of the conflict. Review will be assigned to another IRB member.
    c) When feasible, and not conflicted, the Designated Reviewer for a study is from the PI's (Principal Investigator) home center (e.g., ARC, LaRC, JSC, KSC).
    d) NASA IRB Chair or Designated Reviewer may exercise all authorities of the convened IRB except disapproval. Human Subjects Research may only be disapproved following review by the convened IRB.
    e) The Designated Reviewer may request consultation, additional reviewers with expertise, or may refer the submission to the convened IRB at any time.

f) Administrative modifications (am) to previously approved research may be conducted by the NASA IRB office staff.

2. Eligibility for Expedited Review.

a) **Initial review** of research may occur through expedited review procedures, per 14 CFR 1230.110 and 21 CFR 56.110, provided:

i) the research presents no more than minimal risk to human subjects, and

ii) all procedures fall within the categories of research published by the Secretary of HHS, published in the Federal Register [see Appendix 1].

(1) Note: An activity's inclusion on the list of categories merely means that the activity is eligible for review through the expedited review procedure when the specific circumstances of the proposed research involve no more than minimal risk to human subjects.

iii) Or, for the initial review of research where limited IRB review is a condition of exemption under 14 CFR 1230.104(d)(2)(iii), (d)(3)(i)(c), and (d)(7) and (8).

b) **Modifications to research initially approved via Expedited Review** may be reviewed using expedited procedure provided:

i) All research activities continue to meet the Expedited Research Categories and

ii) When the modifications are implemented, the research continues to pose no more than minimal risk.

iii) Or research for which limited IRB review is a condition of exemption under 14 CFR 1230.104(d)(2)(iii), (d)(3)(i)(c), and (d)(7) and (8).

c) **Modifications to research initially approved by a convened IRB** may be reviewed by the expedited review process provided:

i) Proposed modifications constitute a minor change to previously approved research per 14 CFR 1230.110 (b)(1)(ii), and/or FDA 21 CFR56.110(b)(2), interpreted by the NASA IRB as:

(1) **Minor Modifications (mm)** are those changes to approved research that, in the opinion of the IRB reviewer, neither increases risk nor decreases benefit overall; provided the proposed changes would not negatively impact prior determinations made by the IRB. (see Appendix 2)

(2) If proposed changes include the addition of new procedures, those procedures must fall within the expedited review categories as published in the Federal Register. (see Appendix 1)

ii) **Administrative Modifications (am)** are non-material, administrative or typographic revisions (e.g., training documentation updates, study closures, addition of executed reliance documentation, local ethics documentation, typographic corrections) that may be reviewed and approved by NASA IRB office staff. (See Appendix 2)

iii) **Continuing Review of approved research**: Greater than minimal research, or minimal risk research for which the IRB has determined requires Continuing Review, may be

reviewed through expedited procedures if it meets criteria for Expedited Categories 8 or 9.

(1) Category 8: Continuing review of research previously approved by the convened IRB

(a) 8(a) where (i) the research is permanently closed to enrollment of new subjects; (ii) all subjects have completed all research-related interventions; and (iii) the research remains active only for long-term follow-up of subjects; OR

(b) 8(b) where no subjects have been enrolled and no additional risks have been identified, OR

(c) 8(c) where remaining research activities are limited to data analysis.

(2) Category 9: Continuing review of research, not conducted under an investigational new drug application or investigational device exemption where categories two (2) through eight (8) do not apply but the IRB has determined and documented at a convened meeting that the research involves no greater than minimal risk and no additional risks have been identified.

3. **Expedited Review categories 1-7 do not apply for**:

a) Research involving greater than minimal risk procedures

b) Any study involving human genomic testing

i) NASA policy defines all studies that generate or utilize identifiable genetic information are categorized as "greater than minimal risk." [NPR 7100]

c) When identification of the subjects and/or their responses would easily place them at risk of criminal or civil liability or be damaging to the subjects' reputation, financial standing, employability, etc., unless reasonable and sufficient protections will be implemented so that risks related to invasion of privacy and/or breach of confidentiality are no greater than minimal.

d) For classified research involving human subjects.

Procedures

1) Principal Investigators (PI) request prospective IRB review and approval from the NASA IRB for all new human subjects research and any modifications/changes to approved research through the eIRB.

2) NASA IRB Office responsibilities

a) **Screening/pre-review.** Upon receipt of the submission in the eIRB, the NASA IRB office staff screens the application for completeness, accuracy, and expedited review eligibility.

i) The submission may be returned to the PI for questions, clarifications, or updates. The NASA IRB Office may consult with senior staff, IRB Chairs, and members as needed, particularly with respect to assessments of risks and benefits.

ii) When pre-review is complete, IRB office staff document the following within the eIRB:

(1) Regulatory Oversight (NASA indicated as “Other federal agency”)

(2) Need for any special determinations and waivers

(3) Type of research (e.g., biomedical, SBER)

(4) Additional study features

(a) Clinical Trial – meets either the Revised Common Rule and/or FDAAA clinical trial definition(s)

(b) Inclusion of a Certificate of Confidentiality (CoC)

(c) Collaborative – researchers from outside of NASA are engaged in HSR for the study

(d) Multi-site – involves the same research protocol completed at more than one location (site) (note: rare for NASA studies).

(5) Notes section:

(a) summarize the risk level, any modifications, proposed expedited review category(s) within the eIRB

(b) Non-initial submission pre-review summaries should begin with the modification number to facilitate recordkeeping.

(c) Continuing Review pre-review should include a summary of enrollment, current risk level, proposed expedited category(s)

3) Ancillary Review. Some submissions require additional process(es) to ensure appropriate review.

a) **Crew Office Review:** New studies that include prospective in-flight and/or pre- and post-flight data collection from astronauts and modifications to existing studies where in-flight measures are changing receive an ancillary review by the NASA IRB Crew (CB) Representative.

i) NASA IRB office staff email the Crew Representative with a high-level summary of the study and modifications, as applicable, and link to eIRB submission and request concurrence for approval.

ii) IRB office staff work with the PI to resolve any questions/clarifications raised by the Crew Representative.

b) **Safety Office Review:** Submissions that include equipment or devices not previously reviewed by the NASA Safety Office receive an ancillary review by the NASA IRB Safety Representative.

i) IRB staff request Safety review if there is uncertainty regarding whether equipment or devices have previously been reviewed/cleared.

ii) NASA IRB office staff email the Safety Representative with a high-level summary of the study and modifications, as applicable, and link to eIRB submission and request concurrence for approval.

iii) IRB office staff work with the research team to resolve any questions/clarifications raised by the Safety Representative.

4) **Designated Review:** Following the initial pre-review and completion of any necessary ancillary reviews, the NASA IRB Office forwards the submission to the **Designated Reviewer** for expedited review.

   a) When feasible, and not conflicted, the NASA IRB member from the PI's home center (i.e., ARC, LaRC, JSC, KSC) or designee is assigned as the Designated Reviewer.

      i) Designated Reviewers who have a conflict of interest related to a submission, must inform NASA IRB office staff of the conflict. Review will be assigned to another IRB Member.

   b) The Designated Reviewer may ask the NASA IRB Chair/designee or other qualified NASA IRB Member to assist with review.

   c) Designated Reviewers have access to the materials submitted in the eIRB system including, but not limited to the full protocol, consent document(s), recruitment materials, and data collection materials.

   d) The NASA IRB Chair, designated reviewer, and/or additional ancillary reviewer(s) may provide comments within the in eIRB.

   e) The Designated Reviewer may contact the PI for any clarification needed via "Request for Clarification" within the eIRB.

   f) The Designated Reviewer may choose to complete Worksheet: 431 Expedited Reviewer Checklist. If desired, this checklist may be uploaded into the eIRB under "Submit Designated Review" > "Supporting Documents"

   g) The Designated Reviewer confirms that the research meets ethical principles and standards for protecting research subjects 14 CFR 1230.111.

   h) When satisfied, the Designated Review submits the review through the eIRB system.

5) **Expedited Review.** Expedited review must fulfill the requirements of criteria for approval.

   a) The NASA IRB Chair or Designated Reviewer completes the review applying all applicable approval criteria (14 CFR 1230.111, §_.116, §_.117; and FDA 21 CFR 56.111, 21 CFR 50 Subpart B as applicable).

      i) The requirements for informed consent (§_.116, §_.117) apply regardless of whether research is reviewed by the convened IRB or under an expedited procedure.

   b) If the reviewer finds that the research should not be approved via expedited procedures, it must be referred to the Full Committee for final determinations.

   c) The Designated Reviewer makes one of the following determinations:

      i) **Approved** as minimal risk research. Accepted as written, no changes are required. Criteria for approval are met.

         (1) When an Expedited Review is used for review of a Minor Modification (mm), as defined in Appendix 2, the Designated Reviewer will check 'mm' in the eIRB and briefly summarize the modification.

(2) When an Expedited Review is used for an Administrative Modification (am), as defined in Appendix 2, the IRB office staff/reviewer will check 'am' in the eIRB and briefly summarize the modification.

ii) **Modifications Required to Secure "Approved"** (i.e., Contingent Approval) Minor specific changes are required. The NASA IRB Chair /Designated Reviewer notes the stipulated changes required for approval in eIRB.

(1) Stipulated changes must be directives for which completion may be verified by IRB office staff.

(a) Clarifications or other non-directive requests must be handled directly with the research team through the eIRB via "Request for Clarifications" and not as contingent approval.

(b) Directive stipulations can be checked 'complete' by the NASA IRB office staff. Responses do not need to be reviewed by the NASA IRB Chair or designee.

(c) The NASA IRB Chair or designee may request review of the responses to directive stipulations, in which case the NASA IRB office staff will forward responses to the NASA IRB Chair or designee for final approval.

(d) "Approval date" represents the date that the IRB made the decision to contingently approve the study, not the day the decision is recorded.

(e) "Effective date" is set to the day the modifications are reviewed and accepted by IRB staff.

iii) **Not Human Subjects Research (NHSR)**. Activities described do not meet the definitions for "research" with "human subjects" (14 CFR 1230.102) and do not require IRB oversight.

iv) **Exempt.** The research meets the criteria for an Exempt Determination (14 CFR 1230.104(d)).

d) **Referred for Full Committee Review**. The reviewer may determine that the submission is not expedited and requires review by the convened IRB.

i) Only the Full Committee may disapprove a study if the criteria for NASA IRB approval are not met.

6) Documentation of expedited review is recorded in the eIRB system.

a) The Designated Reviewer documents that the study qualifies for Expedited Review, that the study is minimal risk, and indicates the applicable expedited category(s) within the eIRB.

b) NASA IRB records (within the eIRB) for initial and continuing review by the Expedited procedure must include:

i) Documentation of the risk determination and the specific permissible Expedited Review category(s)

ii) Whether Continuing Review is required and, if applicable, the period of approval.

iii) Rationale for requiring continuing review of research eligible for expedited review must be documented within the eIRB. [§_.115(a)(3)]

c) Additional documentation related to study-specific findings including, as applicable:

i) Waiver or alteration of the informed consent process [§ _.116(f)]. The IRB/Designated Reviewer must find and document:

(1) The research involves no more than minimal risk to the subjects

(2) The research could not practicably be carried out with the waiver or alteration

(3) If the research involves using identifiable private information or identifiable biospecimens, the research could not practicably be carried out without using such information or biospecimens in and identifiable format

(4) The waiver or alteration will not adversely affect the rights and welfare or participants

(5) Whenever appropriate, subjects are provided with additional pertinent information after participation

ii) Waiver of the requirement for documentation of consent [§_.117(c)]

iii) Research for which NASA determines that use of single IRB is not appropriate [§_.114(b)(ii)]

iv) The decision to transition research approved before January 21, 2019 under the pre-2018 requirement to the updated Revised Common Rule should be documented, including the date of the decision to transition [§ _.101(l)(4)]

d) Items approved using the Expedited Review procedure are available for IRB member review:

i) as a report within the eIRB,

ii) within the monthly meeting agenda, and

iii) within monthly meeting minutes.

7) Post Review:

a) Following Expedited Review and documentation within the eIRB, IRB staff:

i) Finalize study materials within the eIRB

(1) MOD -- finalize any materials that are new or updated as part of the current modification

(2) MODCR – finalize all active consent forms and active materials (including new or updated).

ii) prepare the appropriate approval letter, and

iii) distribute approval letters to PI/PI Proxy through the eIRB system.

8) Review Frequency

a) **Pre-2018 Rule**: For federally supported research reviewed and approved by Expedited Review procedures on or prior to January 20, 2019, that has not been transitioned to the Revised Common Rule, the NASA IRB must review ongoing research at least annually, unless the NASA IRB requires more frequent review for a particular protocol.

b) **Minimal Risk research initially approved under expedited categories**: Unless an IRB determines (and documents) otherwise, continuing review of research eligible for expedited review is not required [14 CFR 1230.109(f)(1)(i)].

c) Research originally reviewed by the convened IRB

i) Subjects are not yet enrolled in research (Category 8 (b)). These studies must be reviewed at intervals appropriate to degree of risk, but not less than once per year.

ii) Continuing Review is NOT required for research that has progressed to the point that it involves only one or both of the following:

(1) data analysis, including analysis of identifiable private information or identifiable biospecimens (Category 8(c)) or

(2) accessing follow-up clinical data from procedures that subjects would undergo as part of clinical care (Category 8(a))

d) **FDA Regulated research**: The IRB shall conduct continuing review of research that is covered by FDA regulations at intervals appropriate to the degree of risk, but not less than once per year.

Definitions

Administrative Modifications (am): non-material, administrative or typographic revisions (e.g., training documentation updates, study closures, addition of executed reliance documentation, local ethics documentation, typographic corrections) that may be reviewed and approved by NASA IRB office staff.

FDA (Food and Drug Administration)-Regulated Research: The US FDA regulates clinical studies conducted on drugs, biologics, devices, diagnostics, and in some cases dietary supplements and food additives. All such research studies must be conducted in accordance with FDA requirements for the protection of human subjects and IRBs, regardless of funding source (21 CFR 50 and 56). When FDA regulated test articles are used in research at NASA and funded by another agency, more than one set of regulations may apply. Where regulations differ, NASA IRB will apply the stricter regulation.

Full Committee Review: reviews conducted at convened meetings at which IRB membership requirements [14 CFR 1230.107] are met and a quorum consisting of the majority of the members of the IRB is present.

Human Subjects Research:

Human Subject– a living individual about whom an investigator (whether professional or student) conducting research:

(i) Obtains information or biospecimens through intervention or interaction with the individual, and uses, studies or analyzes the information or biospecimens; or

(ii) Obtains, uses, studies, analyzes, or generates identifiable private information or identifiable biospecimens. [§ _.102(e)]

Research: systematic investigation, including research development, testing, and evaluation, designed to develop or contribute to generalizable knowledge. [§ _.102(i)]

Minimal Risk: the probability and magnitude of harm or discomfort anticipated in the research are not greater in and of themselves than those ordinarily encountered in daily life or during the performance of routine physical or psychological examinations or tests. [§ _.102(j)]

Minor Modifications (mm): changes to approved research that, in the opinion of the IRB reviewer, neither increases risk, nor decreases benefit overall; provided the proposed changes would not negatively impact prior determinations made by the IRB.

Acronyms

ARC – Ames Research Center
CoC – Certificate of Confidentiality (NIH – National Institutes of Health)
COI – Conflict of Interest
FDA – US Food and Drug Administration
FDAAA – Food and Drug Administration Amendments Act of 2007
HHS – US Department of Health and Human Services
HRMRB – Human Research Multilateral Review Board
HRP – Human Research Protections
HSR – Human Subjects Research
IRB – Institutional Review Board
JSC – Johnson Space Center
KSC – Kennedy Space Center
LaRC – Langley Research Center
LSAH – Lifetime Surveillance of Astronaut Health
LSDA – Life Sciences Data Archive
NASA – National Aeronautics and Space Administration
NHSR – Not Human Subjects Research
ORA – Office of Research Assurance (NASA)
OHRP – Office of Human Research Protections (HHS)
PI – Principal Investigator
SBER – Social Behavioral

References

NASA

- 14 CFR 1230
- NPR 7100

HHS Regulations

- Expedited Review Procedures Guidance (2003)

- November 9, 1998 Federal Register: OHRP list of research eligible for expedited IRB review
- SACHRP Advisory Committee, Recommendations to OHRP regarding definition of a minor change in research under 45 CFR 46 and 21 CFR 56; July 20, 2011

FDA Regulations and Guidance

- Expedited review procedures for certain kinds of research involving no more than minimal risk, and for minor changes in approved research: 21 CFR 56.110
- Criteria for IRB Approval of Research: 21 CFR 56 111

Version #	Version Notes:	Originated By:	Approved By:	Date Approved:
v.3.3	• Revised "minor modification" language for clarity – no substantive change to interpretation. • Reclassify non-PI personnel change from a "minor mod" to "admin mod"	J. Ensley Gorshe	M. Covington	20May2026
V.3.2	updates to broken links, formatting updates, addition of acronym section	J. Green/J. Kissling	M. Covington	20Aug2025
v.3.1	Minor revisions to verbiage,	S. Asgar	M. Covington	05Aug2025
v.3	Revised "minor modification" definition. Clarifications to policy regarding when expedited review can be used. Substantive revisions to process details.	J. Ensley Gorshe/J. Green	M. Covington	13Sep2023
v.2		J. Kisenwether	M. Covington	18Jun2021
v.1		J. Christensen	M. Covington	06Jul2020

Appendix 1:

Expedited Review: Categories of Research that may be Reviewed Through an Expedited Review Procedure (1998)

Categories of Research That May Be Reviewed by the Institutional Review Board (IRB) through an Expedited Review Procedure [1]

Applicability

A. Research activities that (1) present no more than minimal risk to human subjects, and (2) involve only procedures listed in one or more of the following categories, may be reviewed by the IRB through the expedited review procedure authorized by 45 CFR 46.110 and 21 CFR 56.110. The activities listed should not be deemed to be of minimal risk simply because they are included on this list. Inclusion on this list merely means that the activity is eligible for review through the expedited review procedure when the specific circumstances of the proposed research involve no more than minimal risk to human subjects.

B. The categories in this list apply regardless of the age of subjects, except as noted.

C. The expedited review procedure may not be used where identification of the subjects and/or their responses would reasonably place them at risk of criminal or civil liability or be damaging to the subject's financial standing, employability, insurability, reputation, or be stigmatizing, unless reasonable and appropriate protections will be implemented so that risks related to invasion of privacy and breach of confidentiality are no greater than minimal.

D. The expedited review procedure may not be used for classified research involving human subjects.

E. IRBs are reminded that the standard requirements for informed consent (or its waiver, alteration, or exception) apply regardless of the type of review--expedited or convened--utilized by the IRB.

F. Categories one (1) through seven (7) pertain to both initial and continuing IRB review.

Research Categories

1. Clinical studies of drugs and medical devices only when condition (a) or (b) is met.
    a. (a) Research on drugs for which an investigational new drug application (21 CFR Part 312) is not required. (Note: Research on marketed drugs that significantly increases the risks or decreases the acceptability of the risks associated with the use of the product is not eligible for expedited review.)
    b. Research on medical devices for which (i) an investigational device exemption application (21 CFR Part 812) is not required; or (ii) the medical device is cleared/approved for marketing and the medical device is being used in accordance with its cleared/approved labeling.
2. Collection of blood samples by finger stick, heel stick, ear stick, or venipuncture as follows:

a. from healthy, nonpregnant adults who weigh at least 110 pounds. For these subjects, the amounts drawn may not exceed 550 ml in an 8 week period and collection may not occur more frequently than 2 times per week; or

b. from other adults and children [2], considering the age, weight, and health of the subjects, the collection procedure, the amount of blood to be collected, and the frequency with which it will be collected. For these subjects, the amount drawn may not exceed the lesser of 50 ml or 3 ml per kg in an 8 week period and collection may not occur more frequently than 2 times per week.

3. Prospective collection of biological specimens for research purposes by noninvasive means.

   Examples: (a) hair and nail clippings in a nondisfiguring manner; (b) deciduous teeth at time of exfoliation or if routine patient care indicates a need for extraction; (c) permanent teeth if routine patient care indicates a need for extraction; (d) excreta and external secretions (including sweat); (e) uncannulated saliva collected either in an unstimulated fashion or stimulated by chewing gumbase or wax or by applying a dilute citric solution to the tongue; (f) placenta removed at delivery; (g) amniotic fluid obtained at the time of rupture of the membrane prior to or during labor; (h) supra- and subgingival dental plaque and calculus, provided the collection procedure is not more invasive than routine prophylactic scaling of the teeth and the process is accomplished in accordance with accepted prophylactic techniques; (i) mucosal and skin cells collected by buccal scraping or swab, skin swab, or mouth washings; (j) sputum collected after saline mist nebulization.

4. Collection of data through noninvasive procedures (not involving general anesthesia or sedation) routinely employed in clinical practice, excluding procedures involving x-rays or microwaves. Where medical devices are employed, they must be cleared/approved for marketing. (Studies intended to evaluate the safety and effectiveness of the medical device are not generally eligible for expedited review, including studies of cleared medical devices for new indications.)

5. Examples: (a) physical sensors that are applied either to the surface of the body or at a distance and do not involve input of significant amounts of energy into the subject or an invasion of the subject's privacy; (b) weighing or testing sensory acuity; (c) magnetic resonance imaging; (d) electrocardiography, electroencephalography, thermography, detection of naturally occurring radioactivity, electroretinography, ultrasound, diagnostic infrared imaging, doppler blood flow, and echocardiography; (e) moderate exercise, muscular strength testing, body composition assessment, and flexibility testing where appropriate given the age, weight, and health of the individual.

6. Research involving materials (data, documents, records, or specimens) that have been collected, or will be collected solely for nonresearch purposes (such as medical treatment or diagnosis). (NOTE: Some research in this category may be exempt from the HHS regulations for the protection of human subjects. 45 CFR 46.101(b)(4). This listing refers only to research that is not exempt.)

7. Collection of data from voice, video, digital, or image recordings made for research purposes.
8. Research on individual or group characteristics or behavior (including, but not limited to, research on perception, cognition, motivation, identity, language, communication, cultural beliefs or practices, and social behavior) or research employing survey, interview, oral history, focus group, program evaluation, human factors evaluation, or quality assurance methodologies. (NOTE: Some research in this category may be exempt from the HHS regulations for the protection of human subjects. 45 CFR 46.101(b)(2) and (b)(3). This listing refers only to research that is not exempt.)
9. Continuing review of research previously approved by the convened IRB as follows:
    a. where (i) the research is permanently closed to the enrollment of new subjects; (ii) all subjects have completed all research-related interventions; and (iii) the research remains active only for long-term follow-up of subjects; or
    b. where no subjects have been enrolled and no additional risks have been identified; or
    c. where the remaining research activities are limited to data analysis.

9. Continuing review of research, not conducted under an investigational new drug application or investigational device exemption where categories two (2) through eight (8) do not apply but the IRB has determined and documented at a convened meeting that the research involves no greater than minimal risk and no additional risks have been identified.

Appendix 2: Administrative Modifications (am) and Minor Modifications (mm)

Administrative Modifications (am) are non-substantive minor changes to study documentation that may be reviewed and approved by NASA IRB office staff.

1) Examples of administrative modifications may include but are not limited to:
   a) CITI training updates
   b) COI form updates
   c) Adding a research sponsor or funding agency clarification and notification memos to the eIRB study file.
   d) Adding reliance/IIA documents
   e) Typographical or *minor editorial corrections* to study documents, including consent forms, recruitment materials, protocols, etc.
   f) Requests for study closure
   g) Change in appropriately credentialed study personnel that is not a change to Principle Investigator (PI)

Minor Modifications (mm): Some proposed modifications to research-related activities approved by the convened IRB may be considered minor (i.e., minor modifications) that do not require review by the convened IRB, per 14 CFR 1230.110(b)(1)(ii).

While federal regulations do not define a “minor change,” the NASA IRB considers those changes to approved research that, in the opinion of the IRB reviewer, neither materially increase overall risk, nor materially decrease benefit; provided the proposed changes would not negatively impact prior determinations made by the IRB. If the minor changes include the addition of procedures, the procedures must be eligible for expedited review.

The reviewer will make the final determination whether the proposed change may be reviewed using expedited procedures. Examples may include but are not limited to:

   a. Change in consent wording does not increase risk or decrease benefit
      i. If the consent change might alter the future/current/past subjects’ willingness to participate, the change **may** require Full Board review (if the study risk is greater than minimal)
   b. Increase/decrease in subject number (a statistically small change or a change that will not alter the overall premise of the study)
   c. Adding monitoring parameters aimed at enhancing subject safety
   d. A new media advertisement
   e. Changes to inclusion or exclusion criteria without increasing risk to subjects
   f. Limited changes in the dosage form (e.g., tablet to liquid) but not route of administration
   g. Extending an observational follow-up period (depending on the nature of the procedures, if any, used to collect the observational data)
   h. Changes in the number of study visits without increase in risk to subjects
   i. Change in remuneration (i.e., payments and reimbursement) to subjects

j. Complement increment mission changes to personnel or data sharing plan

k. Addition of a contingency plan for repeat measures of minimal risk procedures (provided measures are reasonable and do not increase the risks to subjects as currently outlined in the study)